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Name: رمضان علي ابراهيم مرسى National ID: 26004251701419	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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A patient of prostate cancer for Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

MDT FORM

8 / 12 / 2025 G

Name	روفا على ابراهيم	MRN	C. 60
Age	65 y	ID	C. 60
Menopausal status		Phone	0196120779
NRS SCORE		Marital status	married
BMI		Family history	2 Wives
CO morbidity	NAD	REASON OF MDT	عن... ٤٤٠١٧.١٤١٩

Clinical Data

BX: prostatic acinar adenocarcinoma L			
ER:	PR:	HER 2:	Ki67:
Gleson Score 3+4 = 7			
Sonomammography: 30 % of presented Cores			
Mets Workup: Prostatic acinar adenocarcinoma			
Gleson Score 3+3 = 6 ^{TNM} involving 20 % Bone Scan, free			

Recommendations:

- MRI : enlarged prostate 5.1 X 4.1 X 6.8 cm PIRAD 4
- evidence of oval shaped ill margined low T2SI focal lesion Implicating peripheral zone

MDT OUTCOME & PLAN

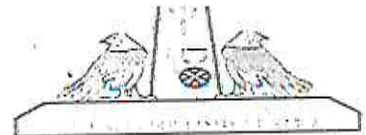
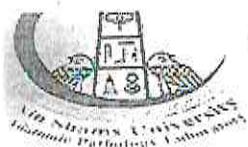
PSA=6 CAP: free

Intermediate Risk Rtu 38F + MPT + 2 side 6 months

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kelly		Nerv 8/12/2025			

Name: رمضان علي ابراهيم مرسى			
MR. : 9465			
Detailed Medical Report			
		السيد	
تجبة طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient was diagnosed with prostatic acinar adenocarcinoma with a Gleason score of 3+4 = 7. Imaging with CT of the pelvis and bone scan was performed, and the PSA was 6. The patient was classified as intermediate risk and was started on Zoladex with concurrent radical radiotherapy. Casodex was administered and completed today, with the next dose of Zoladex scheduled for 28/6. The patient completed radical radiotherapy of 60 Gy in 20 fractions, ending on 14/6/26, with good tolerance. Zoladex was administered today, and a new PSA was obtained. The current status is that the patient is continuing Zoladex therapy as planned.			
تحريراً في ,			
التاريخ :		19-09-2026	
الوقت :			
هنا اسامه عبد الرحيم عبد البر		الطبيب المعالج :	
يعتمد ,			



Patient Name: رمضان علي ابراهيم مرس
IRN: 10210422164673
D.O.B: 25/04/1960
ID No.: 26004251701419

Location: External Clinic
Sex: Male

Surgical Pathology
Specimen

Specimen Id 2508021365
Accession No SU-25-5202
Sample Type TISSUE

Collection Date 02/08/2025 12:55 PM
Received Date 02/08/2025 12:55 PM

Specimen

TRUS guided biopsy from peripheral zone:
I: Right upper prostate biopsy.
II: Right middle prostate biopsy.
III: Right lower prostate biopsy.
IV: Left upper prostate biopsy.
V: Left middle prostate biopsy.
VII: Left lower prostate biopsy.

GROSS DESCRIPTION

I: Two tan tissue cores, each measured 1.3cm long, Totally submitted.
II: Two greyish white tissue cores, each measured 0.8cm long, totally submitted.
III: Two tan tissue cores, each measured 1cm long , totally submitted.
IV: Two tan tissue cores, each measured 1cm long, totally submitted.
V: Two tan tissue cores, each measured 0.8cm long, Totally submitted.
VI: Two greyish white tissue cores measured 0.7cm & 1.5cm long, Totally submitted.

MICROSCOPIC

- Examination of serial section from specimen (I,II,III,VI) revealed cores of prostatic tissue showing hyperplastic bland lined double layered prostatic acini surrounded by intact myoepithelial cell layer.
Some are cystically dilated and showed papillary infoldings and filled with corpora amylacea. Surrounded by hyperplastic fibromuscular stroma entangling few chronic inflammatory cells. No specific granuloma.
No PIN or malignancy.
- Examination of serial sections from specimen (IV,V) revealed cores of prostatic tissue infiltrated by malignant neoplastic growth comprised of well formed and fused acini lined by malignant acinar epithelial cells with enlarged nuclei and prominent nucleoli. Surrounded by mild desmoplastic reaction.

DIAGNOSIS

I: Right upper part of prostate; Tru- cut biopsy:
- Benign prostatic hyperplasia.
- No PIN or malignancy.
II: Right middle part of prostate; tru- cut biopsy:
- Benign prostatic hyperplasia.
- No PIN or malignancy.
III: Right lower part of prostate; tru- cut biopsy:
- Benign prostatic hyperplasia.
- No PIN or malignancy.
IV: Left upper part of prostate, tru- cut biopsy:
- Prostatic acinar adenocarcinoma, Gleason score 7(3+4), group grade II occupying 25% of core

(Handwritten signature)



Patient Name	رمضان على ابراهيم مرس	PID	1021042216 4673	D/T	22-05-2025
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Abdominal and Pelvic Ultrasonography
Suboptimal study due to distension and obesity

- ? **Liver:** It is of average size (12 cm) . It shows bright homogenous echopattern. No focal lesions or dilated biliary radicles.
- ? **PV** is not dilated
- ? **Spleen:** Average size (12 cm) with homogenous echo-texture and no focal lesion.
- ? **Gall bladder:** It shows average wall thickness with no stones or masses. Some comet tail artefacts noted suggesting adenomyosis.
- ? **Pancreas and para-aortic area:** No focal masses or lymphadenopathy.
- ? **Both kidneys:** Average size with normal site, shape, parenchymal thickness and echogenicity. No stones or back pressure changes detected.
- ? No ascites.
- ? **The urinary bladder:** shows average filling with no evidence of intra vesicle masses or stones.
- ? **Pre-voiding volume** 444 cc
- ? **Post voiding volume** 220 cc
- ? **Prostate** is of enlarged size measuring estimated volume 65 cc
- ? No pelvic masses or fluid collection.

Conclusion:

Prostatomegaly with significant post voiding residual volume for PSA correlation

Bright liver likely fatty for clinical and lab correlation.

Query gall bladder adenomyosis for follow up/

***Much Obligated,
Horeya Wahba, MSc***



Patient Name	رمضان علي ابراهيم مرس	PID	1021042216 4673	D/T	13-07-2025
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Multiparametric MRI examination of the prostate

TECHNIQUE: Multiplanar, multisequence imaging include : T2WI ,DWI, pre and post contrast T1WI

FINDINGS:

- Enlarged prostate measures 5.1x4.1x6.8 cm along the three orthogonal planes, with estimated volume 71 ml.
- Evidence of oval-shaped ill-marginated low T2SI focal lesion is seen implicating the peripheral zone along the base of the gland on the left side with corresponding true DW restriction and early post contrast enhancement, it is seen measuring 15x18 mm. The lesion is categorized as PIRADS 4.
- The rest of the peripheral zone shows few faint lenticular T2 hypo intense small areas on the right postero-medial and lateral zones of prostate base with corresponding faint DWI restriction. Also similar triangular areas of faint diffusion restriction bilaterally in mid prostate. These lesions are categorized as PIRADS 3.
- The central zone of the prostate shows multiple central multiple well encapsulated adenomas showing a diffuse patchy heterogeneous signal intensity on T2WI with no corresponding diffusion restriction. (PIRADS 2)
- As regards the Extra-capsular extension: Intact prostate capsule with no lesions seen beyond.

As regards the seminal vesicles: Both seminal vesicles show normal MR appearance.

Patient Name	رمضان على إبراهيم مرس	PID	1021042216 4673	D/T	23-07-2025
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C.T. SCAN OF THE CHEST :-

Axial contiguous thin cuts were taken through the chest after I.V. contrast injection..

Bilateral lower lobar basal GGO with scattered fibroatelectatic bands

Right upper lobe anterior segment small calcific nodule measuring about 2.6 mm

otherwise, Both lungs show normal appearance of its parenchyma with no evidence of abnormal masses, nodules or consolidative patches.

Normal C.T. appearance of the mediastinum vessels.

Multiple subcentemetric mediastinal Lns largest seen pre tracheal and calcific measuring about 7mm .

Patent tracheo bronchial tree with no abnormal bronchial dilatation.

No pleural or pericardial effusion.

Normal cardiac borders and chambers.

IMPRESSION:-

Bilateral lower lobar basal GGO with scattered fibroatelectatic bands

Mostly benign for follow up

C.T. SCAN OF THE ABDOMEN AND PELVIS:-

Axial contiguous cuts were taken through the abdomen and pelvis with per-oral and per-rectal gastrografin, and after I.V. contrast injection.

✍ The liver is of average size displaying normal homogenous CT density with no focal lesion seen.

✍ No extra or intra-hepatic biliary dilatation.

✍ The spleen is of average size displaying normal and homogenous CT appearance with small splenule is seen measuring 6mm

✍ The pancreas is normal .

Patient Name: Ramadan Aly Ibrahim Morsy Patient ID: 1400616
Ref. Phy.: Hasan Shaker Study Date: 30/10/2025

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- 65-year-old gentleman presented with prostatic cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No evidence of bone secondaries could be seen.
- Degenerative changes are noticed at the spine and big joints.

IMPRESSION

- The skeleton is free from metastases.

Much obliged,
Dr. Ahmed NASR, MD
Dr. Alaa Nagi, MD
Nuclear Medicine